

Sezary Syndrome

DEFINITION

Sezary Syndrome (SS) is a leukemic variant of cutaneous T-cell lymphoma (CTCL) that occurs exclusively in adults. It is characterized by the triad of diffuse erythroderma, generalized lymphadenopathy, and circulating malignant T cells with cerebriform nuclei, known as Sezary cells.

CLINICAL MANIFESTATIONS

Clinical findings include severe scaling or fissuring of the palms and soles, alopecia, and onychodystrophy. The erythroderma may be associated with marked exfoliation, edema, and lichenification, and is typically extremely pruritic.

LABORATORY FINDINGS

SS demonstrates histologic features similar to those of mycosis fungoides (MF). According to diagnostic criteria proposed by the International Society for Cutaneous Lymphoma and incorporated into the WHO-EORTC classification, diagnosis requires:

- Skin biopsy findings consistent with CTCL
- Absolute Sezary cell count in peripheral blood ≥ 1000 cells/mm³
- Peripheral blood flow cytometry showing:
 - CD4/CD8 ratio > 10
 - Loss of CD7 expression in $> 40\%$ of T cells (CD7⁻ phenotype)
 - Evidence of a clonal circulating T-cell population

Patients who lack definitive peripheral blood atypia or clonal evidence are classified as having erythrodermic CTCL rather than SS.

TREATMENT AND PROGNOSIS

Compared to patients with patch- or plaque-stage mycosis fungoides, those with SS have a significantly worse prognosis, with a 5-year survival rate of approximately 33%. By the time SS manifests, there is often profound immunosuppression. Many patients die from infectious complications rather than from progressive lymphoma.

Primary Cutaneous CD30⁺ Lymphoproliferative Disorders

Primary cutaneous CD30⁺ lymphoproliferative disorders encompass lymphomatoid papulosis (LyP) and cutaneous anaplastic large cell lymphoma (cALCL). These entities are now recognized as representing a spectrum, with some cases showing overlapping features that make definitive classification based on histomorphology alone challenging.

LYMPHOMATOID PAPULOSIS

Definition

Lymphomatoid papulosis (LyP) was first described by Macaulay in 1968. It is an uncommon, chronic disorder (incidence: 1.2–1.9 cases per 1,000,000) characterized by recurrent, self-healing crops of papules and papulonodules.

Clinical Manifestations

Patients develop disseminated, recurrent papules and papulonodules that may become necrotic or ulcerate centrally.